

DISCHARGE SUMMARY

Patient	Suresh Babu	Age/Sex	67 Years / Male
IP No.	MIO-2024-11532	Ward	Nephrology — Room 305
Admitted	20-Oct-2024	Discharged	27-Oct-2024
Consultant	Dr. Priya Nair, DM Nephrology	Dept.	Nephrology

DIAGNOSIS

- Chronic Kidney Disease Stage 3b — eGFR 32 mL/min (declining)
- Hypertension — poorly controlled, on triple therapy
- Anaemia of Chronic Kidney Disease — Hb 9.1 g/dL
- Hyperuricaemia with Gouty Arthritis — right knee
- Type 2 Diabetes Mellitus — controlled (HbA1c 7.1%)

HISTORY

67-year-old retired government officer. Known T2DM since 2010, Hypertension since 2008, CKD diagnosed 2021. Presented with worsening bilateral ankle oedema, decreased urine output, and right knee pain and swelling for 5 days. Uric acid found elevated on workup.

IMPORTANT NOTES

ALLERGY: Sulfonamides — rash and angioedema. AVOID.

CAUTION: NSAIDs contraindicated — CKD and gout management must avoid NSAIDs.

CAUTION: Contrast media — risk of contrast nephropathy. Pre-hydrate if imaging required.

INVESTIGATIONS

Test	Result	Reference	Status
Serum Creatinine	3.1 mg/dL	0.7–1.2	HIGH ↑↑
eGFR	32 mL/min	> 60	CKD Stage 3b
Blood Urea Nitrogen	54 mg/dL	7–20	HIGH ↑↑
Serum Uric Acid	9.8 mg/dL	3.5–7.2	HIGH — gout
Potassium	5.6 mEq/L	3.5–5.1	HIGH — hyperkalaemia
Sodium	138 mEq/L	135–145	NORMAL
Haemoglobin	9.1 g/dL	13–17	LOW — CKD anaemia
Urine Protein/Creatinine	1.8 g/g	< 0.15	HIGH — proteinuria
HbA1c	7.1%	< 7.0%	BORDERLINE — acceptable

DISCHARGE MEDICATIONS

Medication	Dose	Notes
Amlodipine	10mg OD	Calcium channel blocker
Telmisartan	80mg OD	ARB — renal protective. Monitor K+

Metoprolol	50mg BD	Beta blocker for BP
Furosemide	40mg OD	Loop diuretic — for oedema. Avoid Thiazides
Erythropoietin	4000 IU SC twice weekly	For CKD anaemia
Iron sucrose	200mg IV weekly x4	Iron replacement
Febuxostat	40mg OD	For hyperuricaemia. NOT Allopurinol in CKD stage 3b+
Colchicine	0.5mg OD	Gout prophylaxis. Low dose for CKD
Metformin	STOPPED	Contraindicated in eGFR < 30. Was on 500mg BD.
Insulin Glargine	12 units at night	Switched from Metformin

FOLLOW-UP

- Nephrology in 2 weeks — monitor creatinine, potassium, eGFR trend
- Renal dietitian — low potassium, low phosphorus, low protein diet
- If eGFR drops below 20 — prepare for dialysis planning
- Avoid nephrotoxic drugs: NSAIDs, aminoglycosides, contrast agents